

Chapter 12 (New): The Equity Pillar — Closing Gaps, Not Just Averaging Them

Rural Health Disparities, Racial and Demographic Equity, Social Determinants of Health, the GLP-1 Access Crisis, and Vermont's Equity Architecture

Sources: Vermont Department of Health Health Equity Data Report (January 2025); Oliver Wyman Act 167 Recommendations (2024); Vermont RHT Program Application (November 2025); KFF Medicaid GLP-1 Coverage Analysis (January 2026); CMS BALANCE Model announcement (December 2025); Health Affairs rural health equity research (June 2024); AHS Transformation Reports (2025); Act 68 of 2025.

The Equity Pillar: Why Average Outcomes Are the Wrong Target

The most common failure mode in healthcare quality measurement is optimizing for the average. A system that improves its average readmission rate, average primary care access, or average chronic disease control while allowing — or widening — the gap between its best-served and worst-served populations has not achieved quality improvement. It has achieved statistical improvement while perpetuating or deepening the inequity that produces the worst outcomes.

The Equity pillar of the HTR framework is founded on a different target: every population segment should have access to the conditions that support health, and disparities in access and outcomes that are attributable to race, income, geography, disability, language, sexual orientation, or gender identity should be eliminated, not averaged away. This is not a soft aspiration. It is an analytically precise goal with measurable indicators, specific intervention categories, and accountability structures that Vermont's Act 167 and Act 68 have embedded in law.

Vermont's equity challenge is distinctive enough from the typical urban healthcare equity narrative that it requires explicit treatment before the analytical framework can be applied effectively. Vermont is approximately 94% white — one of the least racially diverse states in the country. Its 4.5% foreign-born population is concentrated in Burlington and a handful of other communities. The equity disparities that drive headlines in Boston, Chicago, or Los Angeles — stark racial gaps in maternal mortality, cardiovascular disease mortality, diabetes management, and access to specialty care — are present in Vermont but at smaller scale and with different geographic distribution. Vermont's dominant equity challenge is geographic: the health and healthcare access gap between Chittenden County (Burlington, the state's economic and medical center) and the rural counties — particularly the Northeast Kingdom — where poverty is higher, providers are scarcer, transportation is unreliable, and broadband is limited.

This does not mean racial equity is unimportant in Vermont. The Vermont Department of Health's 2024 health equity data shows clear disparities: Black adults in Vermont have an 82% health insurance coverage rate compared to 94% statewide. Adults who are Asian, Native Hawaiian or Pacific Islander, Black, or another race are significantly less likely to have a primary care provider than white adults. LGBTQ+ Vermonters, Vermonters with disabilities, and lower-income Vermonters delay care at higher rates due to cost. And Vermont's BIPOC communities face the same structural racism in healthcare settings that affects every state — weight bias, cultural incompetence, language barriers, and documented differential treatment quality.

Vermont's equity agenda must address both the geographic and the demographic dimensions simultaneously — and the analytical framework for doing so is the same in both cases: identify the disparity, trace it to its root cause, and design interventions that address the cause rather than merely measuring the symptom.

Section 1: Vermont's Equity Landscape — What the Data Shows

Geographic Equity: The Rural-Urban Divide

Vermont's most pervasive and consequential health equity disparity is geographic. The Northeast Kingdom — Caledonia, Essex, and Orleans counties — concentrates the state's most severe access, affordability, and outcome disparities:

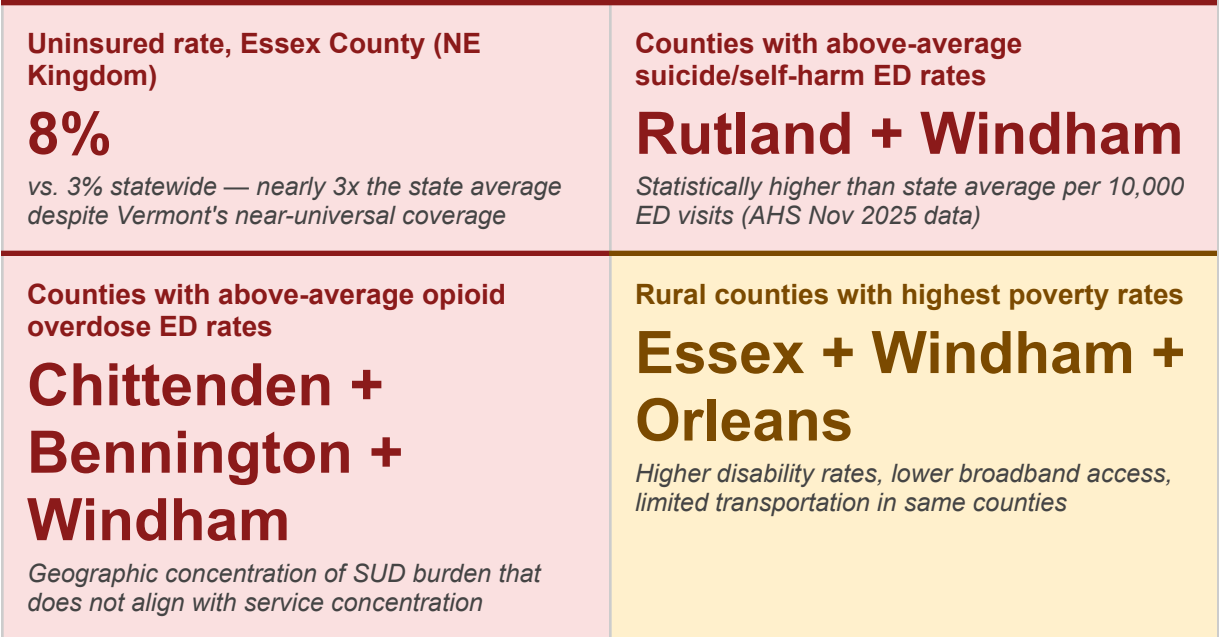


Figure 12.1 — Vermont geographic equity disparities: key indicators. Sources: AHS November 2025 Transformation Report; Vermont RHT Program Application; Vermont Department of Health.

The Northeast Kingdom's geographic isolation creates a compound equity burden that is different in character from urban poverty. A low-income resident of Burlington faces affordability barriers but has access to UVMMC, an FQHC network, mental health services, and public transportation. A low-income resident of Essex County faces the same affordability barriers compounded by 45-minute drives to the nearest primary care provider, inadequate broadband that limits telehealth access, unreliable public transportation that makes scheduled appointments difficult to maintain, and a housing market where the few available rental units are concentrated in towns without healthcare services.

Oliver Wyman's community engagement found that this compound geographic burden produces a distinct pattern of care avoidance: community members described not going to care because the premiums and out-of-pocket costs are too high; long ambulance waits; long and difficult travel to care sites; and long waits in the ED when they do access care. These are not isolated experiences — they are the structural conditions of rural healthcare access in Vermont's most underserved areas.

Racial and Demographic Equity

Vermont's racial equity disparities, while smaller in absolute number than in more diverse states, are documented and consequential. The 2024 Vermont Department of Health equity report shows that statewide, 94% of Vermont adults have health insurance — but only 82% of Black adults. Primary care access follows a similar pattern: while 90% of Vermont adults overall have a personal healthcare provider, the rate for Asian or Pacific Islander adults (81%), Black adults (80%), and adults of another race (79%) is significantly lower. These gaps are not attributable to lack of coverage; they reflect structural barriers to access that persist even when insurance is present.

BIPOC Vermonters, LGBTQ+ Vermonters, Vermonters with disabilities, and lower-income Vermonters were all more likely to delay care due to cost in a 2024 state survey. The same populations were more likely to report their community was not safe for walking — an indicator of the broader social and built environment conditions that shape health. Eight percent of all Vermonters said they did not get medical care because of cost in the prior year; that rate is higher among people with less education, lower income, BIPOC adults, LGBTQ+ adults, and people with a disability.

Vermont's Indigenous population — primarily Abenaki communities — faces specific equity challenges that the state's data systems are not yet adequately equipped to measure. A June 2024 state health equity report documented disparities for the American Indian or Alaska Native population based on 2021-2022 survey data, but Vermont's small and geographically dispersed Indigenous population creates statistical challenges for survey-based measurement. Vermont Public reporting in 2025 found that several Indigenous community organizations had not been in contact with the Health Department, suggesting that outreach and engagement gaps compound the measurement gaps.

The SDOH Burden

Vermont's equity data from the RHT Program application quantifies the SDOH burden that Oliver Wyman identified as both a driver of healthcare utilization and a barrier to healthcare access:

SDOH domain	Vermont statistic	Equity implication
Housing	Rental vacancy rate 3% (healthy market = 5%); half of Vermont renters cost-burdened; per capita homelessness rate second highest nationally	Housing instability is both a health risk factor and a barrier to care — unhoused individuals cannot reliably access scheduled appointments; housing-unstable families face stress that drives both physical and mental health deterioration
Broadband access	80% of rural Vermonters have broadband; 14% of all Vermonters lack access	Broadband gap concentrates in the Northeast Kingdom — the same counties with highest uninsurance and most limited primary care access. Telehealth cannot reach the communities that need it most without broadband.
Food insecurity	9% of Vermonters are food insecure	Food insecurity is a risk factor for diabetes, cardiovascular disease, and mental health conditions — the chronic diseases that drive Vermont's highest healthcare costs. Higher rates in rural counties compound the geographic equity burden.
Transportation	Public transportation limited or unreliable in most rural counties, particularly Northeast Kingdom and Rutland County	Transportation barrier to care is the single most frequently cited access problem in Vermont's community meetings. Without transportation, insurance coverage and provider availability are irrelevant — patients who cannot get to care cannot receive care.
Income	Rural counties (Essex, Windham, Orleans) report higher poverty rates; Northeast Kingdom has highest unemployment	Low income interacts with every other SDOH factor: cost-burdened renters have less money for food, transportation, and healthcare; lower-income workers have less job flexibility to take time off for appointments; financial stress directly affects mental and physical health.

Figure 12.2 — Vermont SDOH burden: key indicators and equity implications. Sources: Vermont RHT Program Application (November 2025); Oliver Wyman Act 167 Report; Vermont Department of Health.

Section 2: The Disparity Root Cause Taxonomy — Why Vermont's Equity Gaps Exist

Identifying a health disparity is the beginning of an equity intervention, not the end. The same disparity in access or outcome can arise from fundamentally different causes — and an intervention that addresses the wrong cause will not close the gap. Vermont's equity strategy must be organized around the root causes of specific disparities, not around the disparities themselves.

Drawing from the HTR Health Equity pillar framework, Vermont's equity gaps fall into five root cause categories. For each category, Vermont has a documented disparity, a traceable mechanism, and a set of interventions — some already underway, some required.

1 Geographic Access Barriers	Mechanisms: Physical distance from providers; transportation infrastructure inadequacy; provider shortage in rural areas; broadband gaps limiting telehealth; limited hours of operation at available facilities. Vermont interventions: COE regionalization with reliable EMS transport; telehealth expansion through RHT Program; broadband infrastructure investment (Oliver Wyman Imperative 1); Blueprint PCMH expansion to all HSAs; community paramedicine.
2 Social Needs Barriers	Mechanisms: Housing instability that prevents sustained engagement with healthcare; food insecurity that drives chronic disease risk; transportation barriers to scheduled care; economic stress that forces delayed care; broadband gaps that prevent telehealth access. Vermont interventions: AHEAD EAST Fund SDOH investment; Blueprint HRSN universal screening with warm handoffs to community resources; AHS HSA Coordinator model linking health and social services; RHT Program investment in community health workers; housing and transportation advocacy as healthcare policy.
3 Clinical Practice Variation	Mechanisms: Documented differential treatment quality by race, ethnicity, disability, and body weight. Oliver Wyman community meetings specifically documented weight bias and fatphobia affecting care access; racial disparities in pain management and referral patterns documented nationally and present in Vermont. Vermont interventions: Provider diversity recruitment and retention; implicit bias training as a condition of Blueprint and AHEAD participation; standardized clinical protocols that reduce provider discretion in ways that produce differential outcomes; patient advisory councils including underrepresented communities; cultural competency standards.
4 Insurance and Coverage Barriers	Mechanisms: Despite Vermont's 97% coverage rate, 8% of Essex County residents are uninsured; 18% of 25-34 year olds are uninsured; income groups between 251-400% of federal poverty line have the highest uninsurance rates despite being ineligible for Medicaid. Even with coverage, high deductibles and out-of-pocket maximums cause care avoidance. Vermont interventions: ACA enrollment outreach in rural communities; FQHC eligibility screening; Act 68 affordability benchmarks; RBP impact on premium reduction that makes insurance more affordable; AHEAD EAST Fund support for FQHCs serving uninsured and underinsured patients.
5 Trust and Engagement Barriers	Mechanisms: Historical and ongoing experiences of discrimination in healthcare settings — documented for LGBTQ+ Vermonters, BIPOC Vermonters, and Vermonters with disabilities. Community meeting feedback specifically cited lack of culturally competent care, stigma, and discrimination as barriers to engagement. Vermont interventions: Community health worker deployment from communities served; HEART program for rural LGBTQ+ Vermonters; BIPOC community partnership grants through VDH Health Equity Capacity Building Program; multilingual patient education materials; patient-centered care training with equity focus; CCBHCs as culturally accessible entry points.

The five root cause categories are not mutually exclusive — a Black rural resident of Windham County may face geographic access barriers, SDOH barriers, and trust barriers simultaneously. Vermont's equity interventions are most effective when they are designed to address multiple root causes for the populations with compound equity burdens, rather than addressing each category in isolation.

Section 3: Rural Equity — Vermont's Defining Challenge

Vermont's rural equity challenge is analytically important for a national audience because it represents a type of health disparity that is often underweighted in national health equity frameworks dominated by urban racial equity narratives. Rural health disparities are not less serious than urban racial disparities — they are different in character, different in mechanism, and require different interventions.

The Health Affairs research on rural health equity in northern New England — directly relevant to Vermont — identifies five principles for addressing rural health equity that align closely with Vermont's transformation agenda. Rural communities need investments that address the social and economic determinants of health, not just the clinical delivery system. Rural health equity requires provider diversity that reflects community demographics and culture. Rural telehealth must be accompanied by the broadband and device infrastructure that makes it genuinely accessible. Rural community organizations must be genuine partners in healthcare design, not passive recipients of state-designed programs. And rural health equity data must be disaggregated at the county and sub-county level — state averages mask the severity of rural disparities.

The Northeast Kingdom: Vermont's Equity Priority

The Northeast Kingdom — Caledonia, Essex, and Orleans counties — is Vermont's clearest and most severe equity priority. It is the most rural part of the state, the most economically distressed, the most underserved by healthcare providers, and the most distant from UVMHC and the Burlington healthcare hub. The three counties that make up the Northeast Kingdom have some of the lowest population densities in the eastern United States, unemployment rates above the state average, high rates of disability, limited broadband coverage, and the highest uninsurance rates in the state.

The healthcare facilities serving the Northeast Kingdom — North Country Hospital in Newport, Northeastern Vermont Regional Hospital in St. Johnsbury, and Copley Hospital in Morrisville for the Lamoille/Orleans border area — are among Vermont's most financially distressed. North Country and Northeastern Vermont Regional are candidates for Tier 3 designation under the Oliver Wyman regionalization blueprint — meaning they may not be able to sustain full inpatient operations in the long term. Any transition away from inpatient services at these facilities, without robust REH conversion and EMS/telehealth investment, would represent a catastrophic equity regression for the communities they serve.

Vermont's AHS must be explicit in the Statewide Strategic Plan that the Northeast Kingdom is an equity priority that constrains the pace and structure of regionalization. The financial logic of consolidating services at RSC hospitals in Burlington, Rutland, or Bennington cannot override the equity obligation to maintain emergency access in the most rural, most isolated communities in the state. The tension between system financial sustainability and rural access equity is genuine, and pretending it is not does not resolve it.

Vermont in Practice: The Northeast Kingdom Equity Constraint

Any Vermont hospital transformation plan that closes emergency services in the Northeast Kingdom without replacing them with services that can actually be accessed by Northeast Kingdom residents — not hypothetically accessible via 90-minute transport to Burlington, but genuinely accessible in the community — fails the equity test that Act 167 and Act 68 have embedded in Vermont law.

The equity constraint does not mean the Northeast Kingdom's hospitals cannot change. It means the sequence matters: investments in EMS capacity, telehealth infrastructure, and community paramedicine must precede or accompany any reduction in inpatient services. The Rural Emergency Hospital designation provides a viable path precisely because it maintains 24/7 emergency access while eliminating the unsustainable inpatient infrastructure. The RHT Program's priority investments in the Northeast Kingdom — broadband, EMS, RPM, telehealth — are equity investments as much as access investments.

The equity test for any transformation proposal affecting Northeast Kingdom hospitals should be specific: after this change, can a 70-year-old resident of Canaan, Vermont who has a heart attack at 2 a.m. receive timely emergency care? Can a person with serious mental illness in Newport access a CCBHC without a two-hour drive? Can a child in Lyndonville see a primary care provider within 14 days? These are not abstract equity metrics. They are the minimum conditions for health equity in rural Vermont.

Section 4: The GLP-1 Access Crisis — The Most Important New Equity Issue

The emergence of GLP-1 receptor agonists — semaglutide (Wegovy, Ozempic) and tirzepatide (Zepbound, Mounjaro) — as transformatively effective treatments for obesity, type 2 diabetes, and cardiovascular disease risk has created what may be the most consequential new health equity issue in American healthcare since the introduction of statins. The drugs work. The evidence is unambiguous: they produce average weight loss of 15-20% of body weight, dramatically reduce cardiovascular events in high-risk populations, and improve glycemic control in type 2 diabetes. For patients at the intersection of obesity, diabetes, and cardiovascular disease — a population concentrated in lower-income and rural communities — GLP-1s could be transformative at the population level.

The equity problem is pricing and coverage. At list prices exceeding \$1,000 per month before rebates, GLP-1s for obesity are financially inaccessible to the vast majority of patients who would benefit from them. As of January 2026, only 13 state Medicaid programs covered GLP-1s for obesity treatment — down from 16 in late 2025, as four states eliminated coverage in response to budget pressure from Medicaid cuts. Vermont Medicaid covers GLP-1s for medically accepted FDA-approved indications but expressly excludes coverage for weight loss under its State Plan. Vermont Medicaid covers GLP-1s for type 2 diabetes management but not for obesity as a primary indication.

The equity consequence is straightforward: GLP-1s are currently accessible primarily to patients with commercial insurance generous enough to cover them, and to patients with type 2 diabetes or cardiovascular disease (who qualify under Medicare and Medicaid for the diabetes and cardiovascular indications). Patients with obesity but not yet diagnosed with type 2 diabetes — many of them lower-income Vermont Medicaid patients in the communities with highest obesity prevalence — cannot access these drugs under current coverage unless they pay full price out of pocket. At \$1,000+ per month, full price is not an option for Medicaid patients.

"Limitations on coverage for obesity drugs in Medicare and Medicaid mean that millions of people who have obesity and might benefit from taking GLP-1s may be unable to access them unless they are able to pay the full cash price out of their own pockets, which would likely be prohibitive for people with Medicaid who must have low incomes to qualify for the program."

— KFF, What to Know About the BALANCE Model for GLP-1s in Medicare and Medicaid, 2026

The BALANCE Model: A Partial Solution

In December 2025, CMS announced the BALANCE (Better Approaches to Lifestyle and Nutrition for Comprehensive hEalth) Model — a voluntary program enabling state Medicaid agencies and Medicare Part D plans to cover GLP-1 medications for obesity by negotiating prices with manufacturers. The model launches in Medicaid as early as May 2026 and in Medicare Part D in January 2027, with a Medicare GLP-1 Bridge program providing \$50/month co-pay access for Medicare beneficiaries beginning July 2026.

BALANCE is a meaningful step toward resolving the GLP-1 equity crisis, but it has three significant limitations that Vermont and every state must navigate. First, manufacturer participation is voluntary — if drug manufacturers do not negotiate with CMS under BALANCE terms, the program produces no coverage expansion. Second, state Medicaid agency participation is voluntary — the four states that just eliminated GLP-1 obesity coverage are unlikely to opt into BALANCE given the budget pressures that drove the elimination. Third, the negotiated prices, while reduced from list prices, may still be expensive for state Medicaid budgets already under pressure from the 2025 reconciliation legislation's Medicaid cuts.

Vermont's DVHA must evaluate BALANCE participation as an equity policy decision, not purely a budget decision. The populations who would benefit most from GLP-1 obesity coverage under Vermont Medicaid — lower-income rural Vermonters with obesity, diabetes risk, and cardiovascular disease risk — are precisely the populations whose healthcare costs are most likely to be reduced by effective obesity treatment. The long-term fiscal argument for coverage is strong; the short-term budget pressure against it is real.

The GLP-1 Equity Implication for Vermont's Transformation

Vermont's transformation narrative must grapple with the GLP-1 equity question explicitly, for two reasons. First, obesity and its metabolic consequences — type 2 diabetes, cardiovascular disease, hypertension, sleep apnea — are major drivers of the high-cost utilization that Vermont's global budget system will be trying to reduce. GLP-1s have demonstrated efficacy in reducing all of these conditions. If GLP-1s become broadly accessible to Vermont's Medicaid population through BALANCE participation, they could accelerate the utilization reduction that global budgets are designed to incentivize — producing population health improvement and healthcare cost reduction simultaneously.

Second, the GLP-1 access gap is the most vivid current example of what the book's Part V identified as the equity reckoning in pharmaceuticals: potentially transformative drugs whose benefits disproportionately accrue to higher-income patients who can afford them, while lower-income patients — who have higher obesity rates, higher diabetes rates, and higher cardiovascular disease rates, partly because of the same SDOH conditions that drive Vermont's equity gaps — cannot access the treatment. Vermont's equity commitment requires active engagement with this issue, not passive monitoring.

Section 5: Vermont's Equity Architecture — What Is Already Built and What Remains

Vermont has more equity infrastructure than most states of comparable size — a product of its near-universal insurance coverage, the Blueprint for Health's SDOH screening mandate, the Health Equity Advisory Commission, the Health Equity Capacity Building Program, and the explicit equity requirements in Act 167 and Act 68. But Vermont also has documented, persistent equity gaps that this infrastructure has not yet closed. The Statewide Strategic Plan's equity section must be honest about both.

What Is Already Operational

- **Universal SDOH screening: Blueprint PCMH practices conduct universal screening for health-related social needs (HRSN) at every patient encounter, using validated tools and generating warm handoffs to Community Health Teams for navigation and resource connection. This is**

the most systematic SDOH screening infrastructure of any state primary care program.

- **Health Equity Capacity Building Program:** Vermont Department of Health grants to community-based organizations working on health disparities, funded through CDC. Organizations working on equity with LGBTQ+ populations, justice-involved individuals, rural communities, and Indigenous communities have received grants through this program.
- **Health Equity Advisory Commission:** A standing body advising state government on health equity policy. Act 68 requires the Health Care Delivery Advisory Committee to consult with the Commission, embedding equity oversight in the transformation governance structure.
- **CCBHC access policy:** Vermont's CCBHCs are explicitly required to serve all patients regardless of age, diagnosis, insurance status, or geographic location — the most equity-protective access policy for behavioral health services.
- **Act 167 and Act 68 equity requirements:** Both acts explicitly include reducing health inequities among their statutory goals and required outcome measures.

What Must Be Built

Gap	What is missing	Required investment	Timeline
County-level equity data infrastructure	Vermont's equity data is primarily state-level, with BRFSS survey data at the county level for major indicators. Sub-county data is largely absent. The Northeast Kingdom's equity burden cannot be managed without granular geographic data.	VHCURES enhancement to enable county-level SDOH and outcome stratification; VITL connectivity to community-based providers who serve underserved populations; integration of VDH equity data with AHS transformation monitoring	2026-2027 (analytics vendor procurement enables this)
Culturally competent workforce	Vermont's clinical workforce is overwhelmingly white and does not reflect the demographic diversity of Vermont's BIPOC, refugee, and immigrant communities. Provider diversity is both an equity outcome and an equity intervention.	Targeted workforce recruitment from underrepresented communities; tuition assistance with equity focus; cultural competency standards in Blueprint and AHEAD participation requirements; provider diversity reporting	Ongoing; RHT workforce investments provide vehicle
GLP-1 Medicaid coverage decision	Vermont Medicaid currently does not cover GLP-1s for obesity as primary indication. The BALANCE Model opt-in decision is an equity policy choice with direct implications for the Medicaid population with obesity.	DVHA evaluation of BALANCE Model participation; budget modeling of long-term cost reduction from obesity treatment vs. short-term coverage costs; legislative advocacy if state plan amendment required	Decision needed by January 2026 (BALANCE opt-in deadline); coverage effective May 2026 if Vermont participates
Northeast Kingdom service resilience	The NE Kingdom has the state's highest equity burden and the most financially distressed hospital infrastructure. REH conversion and telehealth investments	North Country and NE VT Regional transformation plans must include equity impact analysis; broadband investment as prerequisite to	2026 (CCBHC designation); 2027-2028 (EMS regionalization; telehealth); hospital transition decisions by 2028 Strategic Plan

	must precede any service reduction.	telehealth; EMS regionalization funded through RHT Program; CCBHC designation for Northeast Kingdom Human Services (planned July 2026)	
Trust-building with underserved communities	Vermont's BIPOC, LGBTQ+, Indigenous, and rural communities have documented experiences of discrimination and dismissal in healthcare settings. Trust is not built by policy statements; it is built by consistent, culturally respectful engagement over time.	Community health worker deployment from served communities; patient advisory councils with diverse representation; Indigenous community outreach and partnership (VDH engagement with Abenaki communities); HEART program expansion for rural LGBTQ+ Vermonters	Ongoing; requires sustained commitment, not a one-time program

Figure 12.3 — Vermont equity gaps and required investments. Sources: Vermont Department of Health Health Equity Data Report (January 2025); Oliver Wyman Act 167 Report; AHS Transformation Reports; KFF GLP-1 coverage analysis.

Section 6: Equity and the Six-Pillar Framework — How Every Pillar Has an Equity Dimension

A common mistake in equity strategy is treating equity as a separate program — a set of equity-specific interventions that operate alongside the main healthcare transformation agenda. This approach reliably produces equity programs that are underfunded, deprioritized when budgets are tight, and disconnected from the structural changes that actually determine health outcomes.

The correct approach treats equity as a cross-cutting dimension of every pillar — a lens through which every policy decision, payment design, technology investment, clinical model, and operational change is evaluated for its equity implications. The following framework applies the equity lens to each of the six pillars:

Pillar	Equity risk (if neglected)	Equity opportunity (if designed well)
Policy	RBP and global budget design that protects high-price urban hospitals while reducing access in rural and low-income communities. Legislative mandates that apply uniformly without considering the differential impact on small rural hospitals serving high-Medicaid populations.	Act 68's explicit requirement that RBP consider each hospital's community context, payer mix, and social risk factors. CAH-specific regulatory treatment. Equity metrics embedded in monthly AHS reporting. Act 167's five goals including equity as a statutory requirement.
Economic s	Global budgets that incentivize hospitals to avoid high-cost, high-need populations — a classic risk of capitated payment for populations with complex social needs. RBP savings that accrue to commercially insured patients while Medicaid patients' access is reduced.	AHEAD EAST Fund investment specifically in primary care, behavioral health, and long-term care for underserved populations. RBP premium passthrough monitoring that verifies individual and small-group market beneficiaries receive savings. BALANCE Model participation to extend GLP-1 access to Medicaid populations.
Technolo gy	Analytics platforms that measure average performance without disaggregating by race, income, or	VHCURES county-level equity stratification; AI governance framework requiring bias testing across

	geography, hiding disparities in aggregate data. AI tools with biased training data that perform worse for minority populations or elderly rural patients.	demographic subgroups; HIE connectivity requirements that extend to FQHCs and community-based providers serving underserved populations.
Clinical	Blueprint PCMH practices concentrated in higher-income communities with better-resourced practices, leaving underserved communities without PCMH access. CoCM deployment that requires broadband and technology infrastructure unavailable to rural patients.	Blueprint expansion mandate to all HSAs; CCBHC access policy (serve all regardless of insurance status or geography); CoCM deployment using telephone-based options for patients without broadband; CHW deployment from communities served.
Equity	By definition, equity failures in every other pillar accumulate here. The equity pillar fails when geographic access barriers remain despite transformation, when racial disparities persist despite clinical quality improvement, and when SDOH investments are treated as optional rather than foundational.	HSA-level equity monitoring with Northeast Kingdom as priority; SDOH integration across AHS programs; GLP-1 coverage expansion through BALANCE; community trust-building through CHW deployment and patient advisory councils.
Operations	Hospital transformation plans that optimize for financial sustainability without equity impact analysis — e.g., closing inpatient services in rural communities without ensuring alternative access before closure.	Equity impact assessment requirement for all hospital transformation plans; AHS HSA Coordinator model linking health and social services at community level; Northeast Kingdom equity constraint as a binding parameter on regionalization planning.

Figure 12.4 — Equity as a cross-cutting dimension of all six pillars. Sources: Act 67 of 2025; Oliver Wyman Act 167 Report; HTR six-pillar framework.

Section 7: Vermont's Equity Accountability Framework

Vermont's Statewide Strategic Plan must contain a specific equity accountability framework — a set of measurable indicators, geographic stratifications, and intervention commitments that make the equity goal of Act 167 and Act 68 operational rather than aspirational.

The following framework represents the minimum equity accountability structure the Strategic Plan should establish:

Indicator	Stratification required	2028 target direction	Data source
Uninsured rate	By county; by age group (25-34 focus); by income level	Essex County: reduce from 8% toward statewide 3% by 2030	KFF; Vermont Department of Financial Regulation
Primary care access (personal provider rate)	By race/ethnicity; by county	BIPOC adult rate: reduce gap from 79-81% toward 90% statewide average	BRFSS; Blueprint attribution data
Potentially avoidable ED visit rate	By county; by income quintile	Northeast Kingdom: reduce from current rate toward statewide 32.3% (itself a target for reduction)	VHCURES/VUHDD S via AHS analytics vendor
30-day follow-up after MH/SUD ED visit	By county; by race/ethnicity; by insurance type	Windham and Rutland counties: increase	Blueprint via VHCURES

		toward statewide 76%+ target	
SDOH screening completion rate	By HSA; by patient race/ethnicity	Universal screening in all Blueprint PCMHs; document completion rate by population segment	Blueprint program data
GLP-1 access for Medicaid enrollees with obesity	By income; by county	BALANCE Model participation decision by June 2026; coverage effective if participating	DVHA pharmacy claims via VHCURES
Emergency access time (EMS response)	By county; by time of day	Northeast Kingdom: establish baseline; target improvement through EMS regionalization	EMS dispatch data; RHT Program monitoring
Cultural competency standard compliance	By practice type; by HSA	All Blueprint PCMHs: meet minimum cultural competency standards by 2027	Blueprint quality measurement; VDH health equity reporting

Figure 12.5 — Vermont equity accountability framework: indicators, stratifications, and targets. Sources: Act 167; Act 68; VDH Health Equity Data Report (January 2025); AHS Transformation Reports.

Two principles must govern how Vermont uses this accountability framework. First, equity indicators must be disaggregated — not presented as averages. An average primary care access rate that is improving while the BIPOC rate is stagnant is not equity progress; it is equity masking. The Statewide Strategic Plan's equity section must show performance by population segment, not just system-wide averages.

Second, equity indicators must be connected to interventions. A disparity measure without a corresponding intervention commitment is a measurement of failure, not a strategy for success. Each equity indicator in the Strategic Plan should be paired with the specific programs — CCBHC expansion, CHW deployment, RHT broadband investment, BALANCE participation decision — that are designed to move it, with the responsible agency and the expected timeline for observable improvement.

Key Concepts Introduced or Expanded in This Chapter

Term	Definition
Disparity root cause taxonomy	The HTR framework for categorizing the mechanisms that produce health disparities: geographic access barriers, social needs barriers, clinical practice variation, insurance and coverage barriers, and trust and engagement barriers. Each root cause requires different interventions; addressing the wrong cause does not close the gap.
GLP-1 agonists	A class of medications (semaglutide, tirzepatide) with proven efficacy for obesity, type 2 diabetes, and cardiovascular disease risk reduction. Current Medicare and most Medicaid programs do not cover them for obesity as a primary indication, creating an equity gap in which transformative drugs are accessible to commercially insured patients but not to Medicaid patients with the highest obesity burden.
BALANCE Model	CMS's December 2025 voluntary model enabling state Medicaid agencies and Medicare Part D plans to cover

	GLP-1 medications for obesity by negotiating prices with manufacturers. Medicaid launch as early as May 2026; Medicare Part D in January 2027. Voluntary for manufacturers, states, and plans.
Geographic equity	The dimension of health equity concerning disparities attributable to where a person lives — rural versus urban, Northeast Kingdom versus Burlington. Vermont's dominant equity challenge; requires interventions that address geographic access barriers, transportation, broadband, and provider distribution alongside racial and socioeconomic equity.
SDOH (Social Determinants of Health)	The non-clinical conditions that shape health — housing, food security, transportation, economic security, education, social connection, and safety. Oliver Wyman's first imperative (build housing and fix transportation) is an SDOH intervention as much as a healthcare access intervention.
Health Equity Advisory Commission	Vermont's standing advisory body on health equity policy. Required by Act 68 to be consulted by the Health Care Delivery Advisory Committee, embedding equity oversight in transformation governance.
Northeast Kingdom	Caledonia, Essex, and Orleans counties in Vermont's most rural region. Vermont's highest-priority equity geography: highest uninsurance rates (6-8%), most limited provider access, highest unemployment, lowest broadband connectivity, and most financially distressed hospital infrastructure.
HRSN (Health-Related Social Needs) screening	Universal screening for social needs at primary care visits using validated tools, with warm handoffs to Community Health Teams for navigation. Operational throughout Vermont's Blueprint PCMH network; generates SDOH data that feeds equity monitoring when linked to VHCURES.

Sources: Vermont Department of Health Health Equity Data Report, Annual Report to the Vermont Legislature (January 17, 2025); Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (2024); Vermont Rural Health Transformation Program Application (November 2025); KFF, Medicaid Coverage of and Spending on GLP-1s (January 2026); KFF, What to Know About the BALANCE Model for GLP-1s in Medicare and Medicaid (2026); CMS, BALANCE Model announcement (December 23, 2025); CMS, Medicare GLP-1 Bridge demonstration (March 2026); Health Affairs, Reimagining Rural Health Equity (June 2024); Vermont Public, Vermont is trying to survey underrepresented communities about their health (April 2025); Act 68 of 2025; Act 167 of 2022; AHS November 2025 Health Care System Transformation Report.